

[PROVIDER-NAME]

[PROVIDER-NAME]

[PROVIDER-NAME]

[PROVIDER-NAME]

Joint IBD clinic

Our Ref: T FC/VS/ [MED-ID]

[DATE]

Clinic Letter [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Division B

Care Group - Emergency & Specialist Medicine Gastroenterology

Room CG89, G Level, West Wing, Mailpoint [ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME] , [ADDRESS] [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

Diagnoses:

Long-standing Crohn's disease

Right hemicolectomy with ileocolonic anastomosis Newcastle 2008

Further resection 2018

Infliximab primary non-responder

Current Treatment: [NAME] started 3 rd August 2018, low residue diet.

I reviewed Sandra today at the IBD Clinic. I am pleased to say [NAME] is doing extremely well at the moment. I performed a flexible sigmoidoscopy in November which showed really very minor changes. I was delighted to see that there is a very little evidence of active Crohn's disease. I think we should continue with the [NAME] eight weekly for now. [NAME] is using Cholestagel on a regular basis to control her diarrhoeal symptoms. [NAME] does find that when her stool gets a little bit hard [NAME] has some abdominal pain. I suspect this will settle down with time. [NAME] will be seen by the dietitian to try and open up the variety of her diet a little bit more. I would like to see her back in the clinic in four months time.

With best wishes

Yours sincerely

[NAME]

[NAME]

[DICTATION-META]

[PROVIDER-NAME]

Consultant Gastroenterologist

Copies to:

[NAME]

Dear Sam,

I would be grateful if this lady could be added to the data base.